

Dr. A Zubayer Khan

MBBS (DMC),FCPS(Hematology), BCS(Health)

VTP Fellow (Boston), USA

Associate Professor & Head of the Department

Department of Hematology

National Institute of Laboratory Medicine & Referral Centre,



Ahsania Mission

Cancer & General Hospital, Uttara

A Non Profit Organization of Dhaka Ahsania Mission

Name: Mahbubur Age: 32 Yrs Weight: Date: 30/08/2026 ID: 30082604

Diagnosis

- AML Poor Risk

Investigation

- CBC

Rx,

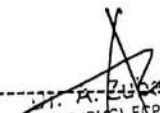
1. Tab. Virux 400 mg

১ + ০ + ১ টি ----- যাওয়ার পরে ----- চলবে

2. Avolac Solution 3.35 gm/5 ml

৩ চা চামচ x ১ বার ----- যাওয়ার পরে, প্রয়োজন মতো, ----- চলবে
পাতলা পায়খানা হলে বন্ধ

2/4/26


Dr. A Zubayer Khan
MBBS (DMC), FCPS (Hematology)
Secretary
Assoc. Prof. & Head of Dept. of Hematology NLMRC
Prof. & Head of Dept. of Cancer & BMT Specialist
Cancer & General Hospital, Uttara

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HELP FIGHT CANCER

Hotline: 10617, 09612310617, 01847359201, 0255092196-7, 0255092140 E-mail: info@amcghbd.org Web: www.amcghbd.org
Fb: www.facebook.com/ahsaniacancer Plot # 03, Embankment Driveway, Sector # 10, Uttara, Dhaka-1230

Ahsania Mission Cancer & General Hospital, Uttara

A Non Profit Organization of Dhaka Ahsania Mission

Hotline: 10617, 09612310817, 01847359201, 0256092198-7, 0256092140

@011220

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Plot # 03, Embankment Driveway, Sector # 10, Uttara, Dhaka-1230

HEMATOLOGY DISCHARGE CERTIFICATE (Indoor)

Charge No	: 2608251944	HIN	: RC2026070278
Patient Name	: MAHBUBUR RAHMAN	Therapy Id	:
Age	: 32 Year	Sex	: Male
Admission Date	: 22/08/2026	Discharge Date	: 26/08/2026
Consultant	1) Dr. A. Zubayer Khan, Consultant, Hematology 2)		

Primary Diagnosis : AML (FLT-TKD positive)

Cytogenetics(PCR/FISH/NGS/Karyotyping) : PCR- FLT TKD detected

IHC/Flowcytometry : Flow - Acute Myeloid leukemia with aberrant expression of CD56

Histopathology : Chronic Large Granulocytic Leukemia(Probable Dx)-Bone Marrow Report
[Possibility of Acute Myeloid Leukemia ?APML can not be ruled out -Bone marrow biopsy-19/06/2026]
[PML-PARA translocation were not detected -Cytogenetics test-24/06/2026]

Co morbidity : Bells palsy

Reason of Admission : Admitted for chemotherapy with C1 IDAC

Chemotherapy Protocol : DA(3+5)(SF:14/7/26), Midostaurin 50mg

Hospital Course : Patient received chemotherapy with C1 IDAC without any chemo related adverse effect> 1unit RBC given .

Treatment History : DA 3+5 (SF:14/7/26)
Tab. Midostaurin

Medication on Discharge	Tab.	Flugal 150 mg	0+1+0	SF: 28/7/26	Continue
	Tab.	Virux 400 mg	0+1+0	Sat, mon,wed	Continue
	Tab.	Omidon 10 mg	1+1+1	Before Meal	7 days
	Syrup	Avolac	3 TSF- 8 hourly	stop if loose motion	Continue
	Supp.	Apit	2 TSF- 12 hourly		7 days
	Tab.	Napa 500 mg	1+1+1	if temp > 100F	
	Tab.	Emistat 8 mg	1+0+1	Before Meal	if vomiting
		Gurgle wih 1% iodine	3 times daily		Continue
		Sonexa eye drop	1 drop in each eye- 6 hourly	SF: 22/08/26	31/08/26
	Inj.	Peg filastin 6miu	1PFS- S./C on 27/8/26 @ 8.00 am		

Next Chemo Date : 11-SEP-26

Next plan : Follow up. Due Date : 29/08/2026

Advice on discharge : Keep Him away from crowded place and visitor restriction.

Do CBC, S. creatinine, SGPT on 29/08/26 and Visit Sir with reports

Dr. Md. Nahid Hossain Khan
Medical Officer, Medical Oncology

Dr. Samira Hassan
Medical Officer, Hematology

Ahsania Mission Cancer & General Hospital, Uttara

A Non Profit Organization of Dhaka Ahsania Mission

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Plot # 03, Embankment Driveway, Sector # 10, Uttara, Dhaka-1230

HEMATOLOGY DISCHARGE CERTIFICATE (Indoor)

Please contact hematology department if any episode of fever, bleeding, loose motion with abdominal pain occurs.

Emergency Contact : 01847359213

IS?

Dr. Md. Nahid Hossain Khan
Medical Officer, Medical Oncology

Dr. Samira Hassan
Medical Officer, Hematology

INVESTIGATION REPORT

Lab/Manual No	: J040790/AML-282	Order No	: OP27/490423
Patient Name	: MD. MAHBUBUR RAHMAN	UHID	: 1397563
Age/Gender	: 32 Y/M	Sample Collected	: 13/08/2026 9:36AM
Referred By	: DR. A ZUBAYER KHAN	Sample Received	: 13/08/2026 10:10AM
Ward/Bed No	: QPD	Report Date	: 13/08/2026 7:48PM
Clinical History	: BONE MARROW	Report Status	: Final

MOLECULAR DIAGNOSTICS LAB

MRD MONITORING AML

Clinical history	: Known case of AML with aberrant expression of CD56
Type of sample	: Bone Marrow in EDTA (Post induction)
Sample quality	: Adequate (As evidenced by presence of mast cells and HSC >100 events)
Events acquired	: ~19,00,000 (19 lac; due to low cell count)
Gating strategy	: SSC/FSC, SSC/CD45, CD34/CD117 and Boolean gating strategy to assess residual blast cells.

RESULT

AML-Measurable Residual diseases are **DETECTED** (0.12%).

DESCRIPTIVE SUMMARY

On CD34/CD117 gating analysis, 0.12% abnormal myeloid blasts are seen which are dim for CD45 and show moderate side scatter (Blast window). These blasts express bright CD56; moderate CD13, CD33, CD38, CD54, CD117; downregulated HLA-DR and negative for CD4, CD5, CD7, CD14, CD15, CD19, CD25, CD34, CD36, CD64, CD123. These critical cells aberrantly express bright CD56.

MRD Calculation:

Total leukocytes events	: 1,042,804
Suspected critical population events	: 1,238
MRD (%)	: 0.12%

Markers performed :

Tube 1: CD45, CD34, CD38, CD117, CD4, CD5, CD7, CD19, CD13, CD33, CD54, HLA-DR

Tube 2: CD45, CD34, CD38, CD117, CD14, CD15, CD25, CD36, CD56, CD64, CD123, HLA-DR

Note:

1. AML-MRD value of $\geq 0.1\%$ is considered as MRD positive.
2. Limit of detection of this assay is 0.01%

MD ABU HASAN
Chief Scientific Officer

Dr. MST RUMMANA RAHIM
MBBS, MPhil (Microbiology) M.Sc (Biotech)
ASSOCIATE CONSULTANT

Dr. MD MIZANUR RAHMAN
MBBS, PhD (Immunology)
SENIOR CONSULTANT - MOLECULAR DIAGNOSTICS

(BMDC No: A23462)

Prepared by Dr. MD MIZANUR RAHMAN

Page 1 of 2

INVESTIGATION REPORT

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Patient Name	: MD. MAHBUBUR RAHMAN	UHID	: 1397563
Age/Gender	: 32 Y/M	Sample Collected	: 13/08/2026 9:36AM
Referred By	: DR. A ZUBAYER KHAN	Sample Received	: 13/08/2026 10:10AM
Ward/Bed No	: OPD	Report Date	: 13/08/2026 7:48PM
Clinical History	: BONE MARROW	Report Status	: Final

MOLECULAR DIAGNOSTICS LAB

MRD MONITORING AML

Please correlate with clinical features, peripheral blood findings, bone marrow morphology and cytogenetic/molecular studies.

Advice: Follow up.

Test platform:

Instrument : 12 color, 3 laser BD FACS Lyric (Becton Dickinson)

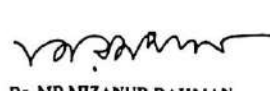
Software : BD FACS Suite software

Method : Bulk lysis followed by direct antibody staining.

— End of Report —


MD ABU HASAN
Chief Scientific Officer


Dr. MST RUMMANA RAHIM
MBBS,MPHI(Microbiology)M.Sc(Biotech)
ASSOCIATE CONSULTANT


Dr. MD MIZANUR RAHMAN
MBBS, PhD (Immunology)
SENIOR CONSULTANT - MOLECULAR DIAGNOSTICS

(BMDC No: A22462)

Prepared By Dr. MD MIZANUR RAHMAN

Page 2 of 2



Ahsania Mission

Cancer & General Hospital, Uttara

A Non Profit Organization of Dhaka Ahsania Mission

Bone Marrow Aspiration Report

Lab Ref: 1499/A/26	Hematology Clinic
Name: Mahbubur Rahman	Age: 32 yrs
Bill. No: 260810011136	Date: 12/08/26

History of AML - FLT3 positive , received induction by DA 3+7.

PBF shows mild normocytic normochromic anaemia

Cellularity is mildly increased with decreased M:E ratio. Erythropoiesis is hyperactive.. Granulopoiesis is also normal and matured with <5% blasts . Megakaryocytes are normal and matured. Other cell lines are unremarkable.

Comment: Suggestive of AML in bone marrow remission with erythroid hyperplasia .

Seen by

Dr. A Zubayer Khan

FCPS (Hematology)

Specialist Hematologist ,AMCGH

HELP FIGHT CANCER

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Fb: www.facebook.com/ahsaniacancer Plot # 03, Embankment Driveway, Sector # 10, Uttara, Dhaka-1230

אנחנו מודים לך על כל מה שאתה עושה בשבילנו.

E-mail: info@amcglb8.org, Web: www.amcglb8.org, Fax: www.facebook.com/dwightlagancor
 P.O. # 02, Emburytoners Drive, Sec 101 # 10, Ufford, Ohio-43080

411275

Charge No	2008040348	MRN	RC2020070278
Patient Name	MAHBUBUR RAHMAN	Therapy Id :	
Age	32 Year	Sex	Male
Admission Date	06/07/2026	Discharge Date : 06/08/2026	Discharge : with advice
Consultant	1) Dr. A. Zubayer Khan, Consultant, Hematology 2)		

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are generated report, Developed by : IT & MIS dept., AMCGH::: Page 1 of 2 ::::Generated on :06-08-26 04:11:0

* Afrin Nasal Drop 2-2 ~~times~~ 2-4 times   

HELP FIGHT CANCER



Ahsania Mission Cancer & General Hospital

A Non Profit Organization of Dhaka Ahsania Mission

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Plot #.03, Embankment-Driveway, Sector #10, Uttara, Dhaka-1230

TEST RESULTS

RE



UNIQUE PATIENT ID

TEST NAME
AML MIA

HEMATOLOGY DISCHARGE CERTIFICATE (Indoor)

Next plan : Follow up Due Date : 12/08/2026

Advice on discharge : Plan-Repeat Fungas 1,3 beta D glucan ag test BIRDEM on 12/08/26 ,

Ensure PICC line Dressing .

Keep Him away from crowded place and visitor restriction.

Do CBC, S. creatinine, SGPT, S. Electrolyte, on 12/08/26 and Visit Sir with reports

Please contact hematology department if any episode of fever, bleeding, loose motion with abdominal pain occurs.

Plan: Bone marrow for MRD on 12/8/26

Emergency Contact : 01847359213(Whatsapp Available)

BMAS Procedure Charge.
Bone Marrow Study.

Dr. A. Zubayer Khan
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Specialist Hematologist
Assoc. Prof. & Head of Dept. of Hematology, NILMRC
Blood Disease, Blood Cancer & BMT Specialist
Ahsania Mission Cancer & General Hospital, Uttara

N/A
N/A, N/A

Adv

Admission in Hematology
ward/DC on 17.8.26
for Short/Long Chemo.

for C, Hi DAE + mido
IDAE
after confirmation.

For Dr. A. Zubayer Khan
16/8/26
Dr. A. Zubayer Khan
MBBS (DMC), FCPS (Hematology)
Specialist Hematologist
Assoc. Prof. & Head of Dept. of Hematology, NILMRC
Blood Disease, Blood Cancer & BMT Specialist
Ahsania Mission Cancer & General Hospital, Uttara

Dr. Mazrana Akter Dolon
Medical Officer, Hematology

3092140
ok.com/ahsanianand
76-1230



ME-2256

UNIQUE PATIENT ID : 890181

Case ID : 26010175136
Patient Name : Mr. MAHBUBUR RAHMAN
Age/DOB/Sex : 32 Years / / Male
Hospital Name : Axon Health Care
Physician Name : Assoc. Prof. Dr. Zubayer Khan
Registration On : 11-Jul-2026 09:51
Collection On : 09-Jul-2026 11:00
Reported On : 15-Jul-2026 18:58
Process AT : CORE-Gurugram
Ref ID :
Sample Type : Peripheral Blood
Report Status : Final

TEST NAME

AML Multiplex Basic (BCR - ABL, PML RaRa, AML ETO, INV 16, NPM1, FLT3, KIT)

SPECIMEN INFORMATION

Peripheral Blood

CLINICAL HISTORY

The overall Immunophenotype findings of specimen provided are compatible with Acute Myeloid Leukemia (AML) with aberrant expression of CD56.

METHODOLOGY

Real Time Polymerase Chain Reaction + Gel electrophoresi

RESULTS

GENES COVERED	CHROMOSOMAL ALTERATION	RESULTS
PML - RARA	t(15;17)(q24.1;q21.1)	Not Detected
AML1-ETO	t(8;21)(q22;q22)	Not Detected
CBFB-MYH11	inv(16)(p13;q22)	Not Detected
BCR-ABL1	t(9;22)(q34;q11) (Major)	Not Detected
BCR-ABL1	t(9;22)(q34;q11) (Minor)	Not Detected
GENES COVERED	ALLELE STATUS	RESULTS
FLT3-ITD*	Wild Type	Not Detected
FLT3-TKD (D835Y/I836)	Mutant	Detected
NPM1	Wild Type	Not Detected
c-KIT (Exon 17)	Wild Type	Not Detected

TEST ATTRIBUTES

- Qualitative analysis performed through Real Time PCR
- FLT3- ITD detection through RT-PCR followed by Gel Electrophoresis*

Disclaimer: This test is performed using in-house developed assay for AML Multiplex Panel. The assay is designed to perform the reactions at the specified analytical sensitivity given that the template DNA is not heavily fragmented and does not contain materials that could inhibit the amplification reaction.



Shivani

Dr. Shivani Sharma
DCP, DNB, FRCPath (London)
Reg. No. 1806

Rahul

Dr. Rahul Katara
Ph.D.

Sanjay

Dr. Sanjay Kumar
Ph.D.



UNIQUE PATIENT ID : 890181

Case ID : 890181
 Patient Name : Mr. MAHABUBUR RAHMAN
 Age / Sex / DOB : 52 Years / Male
 Hospital Name : Asian Health Care
 Physician Name : Assoc. Prof. Dr. Zubair Khan
 Registration No. : 11 Jul 2025 09:41
 Collection On : 09 Jul 2025 11:00
 Reported On : 10 Jul 2025 13:43
 Project ID : CORE-00709301
 Ref ID :
 Sample Type : Peripheral Blood
 Report Status : Final

TEST ATTRIBUTES

LIST OF DETECTABLE VARIANTS AND LIMIT OF DETECTION

COMPONENTS	VARIANT DETECTED	LIMIT OF DETECTION (LOD)
PML RARA	BCR1, BCR2, BCR3	LOD is equal to 10 copies of fusion transcript per PCR
AML1-ETO (t(8;21) (q22;q22)) &	RUNX1-RUNX1T1	
CBFB-MYH11 (inv (16) (p13;q22))	Type A, E, D	
BCR ABL1	E13a2 & e14a2 (P210), e1a2 (p190)	is equal to 2% mutant alleles in background of 98% wild type alleles
FLT3 Internal Tandem Duplication (ITD) & Tyrosine kinase domain (TKD)	TKD(D835), ITD	
NPM1	Type A,B,D	
C-KIT	D816V	

COMMENTS

PML RARA	- Acute promyelocytic leukemia (APL) is a particularly aggressive subtype of AML, comprising approximately 10% of AML cases - The translocation of the PML gene on chromosome 15 to the RARA gene on chromosome 17 [ie, t(15;17) (q24.1;q21.1)] produces a PML-RARA fusion gene - Based on PML breakpoint location, the PML RARA transcripts subtype breakpoint cluster region (BCR)- BCR 1, BCR 2 (long transcript) BCR 3(Short Transcript type) may be formed - For diagnostic identification of PML RARA in cases of Acute Promyelocytic Leukemia - To assess molecular resistance & predict response to treatments containing ATRA and / or ATO
AML1- ETO	- AML1 (RUNX1 - Runt related Transcription Factor 1) is fused with ETO (RUNX1T1) in the t(8;21)(q21;q22) translocation - The AML1-ETO fusion transcription factor is generated by the t(8;21) translocation, which is present in approximately 4%-12% of adult and 12%-30% of pediatric acute myeloid leukemia (AML) patients - The (8;21) translocation is associated with about 40% of cases of M2 AML with karyotypic abnormalities and represents the most frequent chromosomal anomaly in leukemia (18-20%) - For prognostic evaluation - Presence of this translocation is associated with a favorable prognosis



MC-2256

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 Age/DOB/Sex : 32 Years / / Male
 Hospital Name : Axon Health Care
 Physician Name : Assoc. Prof. Dr. Zubayer Khan
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 Reported On : 15-Jul-2026 18:58
 Process AT : CORE-Gurugram
 Ref ID :
 Sample Type : Peripheral Blood
 Report Status : Final

CBFB- MYH11	- The fusion gene CBFB/MYH11 results from a pericentric inversion of chromosome 16, inv(16)(p13.1q22), or less commonly from a t(16;16)(p13.1;q22) - This rearrangement is found in 6-8% of adult de novo AML cases and associated with favorable prognosis - For diagnostic identification of AML having morphological, immunophenotypic or clinical features strongly suggestive of M4eo FAB subtype
BCR ABL	- The Philadelphia chromosome, corresponding to the BCR ABL1 rearrangement, is found in 0.5-3 % of all acute myelogenous leukemia - BCR-ABL-positive acute myeloid leukemia (AML) is a rare subtype of AML that is now included as a provisional entity in the 2016 revised WHO classification of myeloid malignancies - AML with BCR-ABL1 rearrangement is a rare de-novo AML that may benefit from therapies that entail tyrosine kinase inhibitors - The most common BCR-ABL transcripts (p190 and p210) are nearly equally distributed. The prognosis of BCR-ABL+ AML seems to depend on the cytogenetic and/or molecular background rather than on BCR-ABL itself
NPM1	Isolated NPM1 mutation, which localizes to the cytoplasm, confers a higher complete response (CR) rate and improved event-free survival (EFS) and OS compared with patients who are NK-AML and wild-type NPM1, resulting in outcomes similar to patients with favorable cytogenetics (eg, CBF AML) Mutated NPM1 is associated with favorable prognosis without FLT3-ITD or with the low allelic ratio of FLT3-ITD
FLT3	- FLT3-ITD(internal tandem Duplication) mutations occur in approximately 30% of cases and FLT3-TKD (tyrosine kinase domain) mutations in approximately 10% of patients - Negative prognostic influence of FLT3-ITD in patients with AML, resulting in shorter remission durations (eg, decreased disease-free survival [DFS] in patients with a CR and poorer survival outcomes compared with patients who have wild-type FLT3 - The FLT3-TKD mutations predominantly occur independently of FLT3-ITD, and most frequently involve mutations in the D835 residue of a TKD
KIT	- KIT mutations have been reported in approximately 20% of patients with CBF AML - KIT is a receptor tyrosine kinase involved in proliferation, differentiation, and survival

CORE



UNIQUE PATIENT ID : 890181

* The presence of RT inhibitors in some developing countries is generally associated to the introduction of a water program.

[illegible]

Dr. Shivan Shivan
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Reg. No. 10564 of 2

Dr. Robert K. Korte
Ph.D.

En. Sander Kabaner
Ph.D.

CORE DIAGNOSTICS



UNIQUE PATIENT ID : 890181

Case ID : 26010175136
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REFERENCES

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- Troussard X, Rimokh R, Valensi F, Leboeuf D, Fenneteau O, Guitard AM, et al. Heterogeneity of t(1;19)(q23;p13) acute leukemias. French Haematological Cytology Group. Br J Haematol. 1995;89(3):516–26. doi: 10.1111/j.1365-2141.1995.tb08357.
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- Pui CH, Frankel LS, Carroll AJ, Raimondi SC, Shuster JJ, Head DR, Crist WM, Land VJ, Pullen DJ, Steuber CP: Clinical characteristics and treatment outcome of childhood acute lymphoblastic leukemia with the t(4;11)(q21;q23): A collaborative study of 40 cases. Blood 77:440, 1991

Comprehensive Hematological Malignancy Panel Report



Specimen Type		Case Number:	26010175136
EDTA, Peripheral blood		Patient Name:	Mahbubur Rahman
		Age/Sex:	32 Yrs/Male
		Patient Location:	Dhaka
		Hospital Name:	Axon Health Care
		Physician Name:	Prof. Dr. Zubayer Khan
		Date & Time of Reporting:	22/07/2026 12:33 Hrs
Specimen Collection Date & Time	Date & Time of Accessioning		
09/07/2026 00:00 Hrs	11/07/2026 09:51 Hrs		

TEST INFORMATION

The **Comprehensive Hematological Malignancy Panel** aims to determine relevant **mutations** across **40 key DNA genes** and **fusions** across **29 driver genes**, associated with major **hematological disorders** (acute myeloid leukemia (AML), myeloid dysplastic syndrome (MDS), myeloproliferative neoplasms (MPN), chronic myeloid leukemia (CML), chronic myelomonocytic leukemia (CMML), juvenile myelomonocytic leukemia (JMML) and Acute lymphoblastic leukaemia (ALL), by next generation sequencing (**NGS**).

CLINICAL HISTORY

Outside FCM reported Acute Myeloid Leukemia (AML) with aberrant expression of CD56. AML Multiplex Basic (BCR - ABL, PML RaRa, AML ETO, INV 16, NPM1, FLT3, KIT) is positive for FLT3-TKD (D835Y/I836).

RESULTS SUMMARY

– No pathogenic mutation/fusion detected.

VARIANTS OF UNKNOWN FUNCTIONAL/THERAPEUTIC SIGNIFICANCE (VUS)

Gene (Exon) Transcript	Variant	VAF
-	-	-



Comprehensive Hematological Malignancy Panel Report

Case Number: 26010175136

Patient Name: Mahbubur Rahman

Ordering Physician Name: Prof. Dr. Zubayer Khan

ADDITIONAL INFORMATION

Test details: Comprehensive Hematological Malignancy Panel is a multibiomarker NGS assay that enables mutations across 40 key DNA genes and fusions across 29 driver genes relevant to major hematological disorders. The test utilizes Oncomine Myeloid Research Assay, which is based on the proven Ion AmpliSeq technology. This assay allows concurrent analysis of DNA and RNA to simultaneously detect multiple types of variants, including hotspots, single-nucleotide variants (SNVs), indels and gene fusions:

- **Hotspot genes:** ABL1, BRAF, CBL, CSF3R, DNMT3A, FLT3, GATA2, HRAS, IDH1, IDH2, JAK2, KIT, KRAS, MPL, MYD88, NPM1, NRAS, PTPN11, SETBP1, SF3B1, SRSF2, U2AF1, WT1
- **Full genes:** ASXL1, BCOR, CALR, CEBPA, ETV6, EZH2, IKZF1, NF1, PHF6, PRPF8, RB1, RUNX1, SH2B3, STAG2, TET2, TP53, ZRSR2
- **Gene fusions:** ABL1, ALK, BCL2, BRAF, CCND1, CREBBP, EGFR, ETV6, FGFR1, FGFR2, FUS, HMGA2, JAK2, KMT2A (MLL), MECOM, MET, MLLT10, MLLT3, MYBL1, MYH11, NTRK3, NUP214, PDGFRA, PDGFRB, RARA, RBM15, RUNX1, TCF3, TFE3
- **Expression genes:** BAALC, MECOM, MYC, SMC1A, WT1
- **Expression control genes:** EIF2B1, FBXW2, PSMB2, PUM1, TRIM27

Methodology and variant assessment:

DNA and RNA are isolated from the blood sample and quantified using Qubit Fluorometer. Extracted DNA/RNA are used for target amplification. The amplified product is subjected to adapter ligation and barcoding for specific library preparation. The generated high quality library is subjected to next generation sequencing (NGS) on the ION S5 sequencing platform. The output sequences are aligned to the human reference genome hg19 (GRCh37). The alignments and variant calling is done using the ION S5 torrent server. Variants are identified and interpreted using Ion Reporter Software. The identified variant(s) is(are) annotated according to HGVS sequence variant nomenclature. Multiple in-silico predictors, such as SIFT, PolyPhen, MutationTaster, NNSPLICE, and ASSP etc. are used for variant impact on the protein function. Population and literature databases such as dbSNP, genome Aggregation Database (gnomAD), COSMIC, ClinVar, HGMD and PubMed etc are used for variant summary and classification. Variants are labelled as per Standards and Guidelines for the Interpretation and Reporting of Sequence Variants in Cancer, by Association for Molecular Pathology (AMP) with liaison representation from the American College of Medical Genetics and Genomics (ACMG), American Society of Clinical Oncology (ASCO), and College of American Pathologists (CAP).

For the Index patient, on target was 93.60% with a (67.22%) 91.69% Target base coverage at (500X) 100X.



Aditi Aggarwal
Aditi Aggarwal, Molecular Scientist

Dr. Rahul Katara
Dr. Rahul Katara, Ph.D.

Ankita Jaiswal
Dr. Ankita Jaiswal, MD, PDCC
Reg. No. HN17104



Comprehensive Hematological Malignancy Panel Report

Case Number: 26010175136

Patient Name: Mahbubur Rahman

Ordering Physician Name: Prof. Dr. Zubayer Khan

ADDITIONAL INFORMATION

Limitations and disclaimer:

Despite all precautions taken, the error (administrative and technical) associated with these types of molecular diagnostic tests can be as high as 1% to 2%. Rare polymorphisms may be present that could lead to false negative or false positive results. The quality of sequencing and coverage varies between regions; many factors such as homopolymers, GC-rich regions etc. influence the quality of sequencing and coverage. This may result in an occasional error in sequence reads or lack of detection of a particular genetic variant. Furthermore, A negative (wild type) result does not rule out the presence of a mutation or rearrangement resulting in targeted fusion, that may be present but below the limits of detection of this assay. Variants that have not been confirmed by an independent analysis could represent technical artifacts. Not all variants detected may be listed in the report. Inclusion of variants is dependent upon our assessment of their clinical significance. Additionally, the presence of a mutation may not be predictive of response to therapy in all patients. The selection of any potential treatment/course of action based on this report rests solely within the decision and judgment of the treating physician and patient. Decisions on patient care should be based on the independent medical judgment of the treating physician based upon all available clinical information, according to the applicable standard of care and should not be based solely on the tests and information contained in this report. Accurate interpretation of this report is dependent on provided detailed clinical history of the patient. In the event of unavailability of detailed clinical history, the lab cannot guarantee the accuracy of the interpretation. The results and interpretation are based on current knowledge and might change in the future. Some findings listed in this report may be based on pre-clinical studies or studies not in the given patient's tumor type.



Aditi Aggarwal
Aditi Aggarwal, Molecular Scientist

Rahul Katara
Dr. Rahul Katara, Ph.D.

Ankita Jaiswal
Dr. Ankita Jaiswal, MD, PDCC
Reg. No. HN17104



Scan to Connect

If you have any questions about this report or would like to have a conversation about the test results,
please feel free to reach out to us at

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2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory.
3. The reported results are for information and are subject to confirmation and interpretation by the referring doctor.
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406, Udyog Vihar, Phase III, Gurugram, Haryana - 122016
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BIRDEM GENERAL HOSPITAL

Bangladesh Institute of Research and Rehabilitation
In Diabetes, Endocrine and Metabolic Disorders

of the
Bangladesh

LABORATORY SCIENCE DIVISION

DEPARTMENT OF MICROBIOLOGY PROFESSOR FARIDA HUQ LABORATORY

MR Date : 30/07/2026	Receiving Date : 30/07/2026	Reporting Date : 30/07/2026 07:11 PM
MR No : 26071103492	Lab No : 47606	Ud. Lab. No : 46702
Name : Mahbubur Rahman	Age : 32 Y	Sex : M
Ref. By : n/		
Specimen : Serum		Counter : BI
Investigation : Fungus (1-3) beta D Glucan Ag		
Method : Fluorescence Immunochromatography (Lateral flow assay)		

TEST RESULT

TEST	UNIT	RESULT	REFERENCE RANGE
Fungus (1-3) beta D Glucan Ag	pg/mL	109.3	Negative <80 Positive = or>100 Indeterminate: 80 - 100

Interpretation:

- A) If the result is negative but a fungal infection is still suspected, retesting is recommended.
- B) Positive results should be used in conjunction with clinical symptoms, medical history and other findings.
- C) If the result is indeterminate, continuous testing and observation are recommended.
- D) The sensitivity and specificity of Fungus (1-3) beta D Glucan is 67% to 90% and 66% to 86% respectively in patients with proven invasive fungal infection.
- E) The assay should be use in conjunction with other diagnostic procedures such as microbiological culture, histological examination of biopsy samples and radiographic evidence.

Limitations:

- A) Fungus (1-3) beta D Glucan will be decomposed or neutralized by antibodies, resulting in false negative result.
- B) Single test result cannot be considered as the only basis of disease diagnosis. To achieve the diagnosis, test results can be used as a reference combining clinical s/s and other testing results.
- C) The kit can not be used to distinguish fungal species, mucormycetes and cryptococcus.
- D) False positive result may be seen in recent hemodialysis; administration of fractionated blood products; exposure of high amount of glucan containing gauze or swabs during surgery; bacterial septicemia; anti tumor drugs and use of various antibiotics.


Checked by
Subarna Dutta
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M.Sc in Microbiology (SUB)

Sr. Research Officer


Lab. in Charge
Dr. Lovely Barai
MBBS, FCPS (Microbiology), PhD
(DU)

Professor and Head

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Tel: 88-02-41060501-24, Fax: 880-02-41060476, E-mail: dg@birdembd.org, dg_birdem@dab-bd.org



An enterprise of the
Diabetic Association of Bangladesh

BIRDEM GENERAL HOSPITAL

Bangladesh Institute of Research and Rehabilitation
In Diabetes, Endocrine and Metabolic Disorders

LABORATORY SCIENCE DIVISION

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overcar
HOSPITAL DH
Transforming
Lab / Manual
Patient v
Page / 1
9/2

DEPARTMENT OF MICROBIOLOGY PROFESSOR FARIDA HUQ LABORATORY

MR Date : 30/07/2026	Receiving Date : 30/07/2026	Reporting Date : 30/07/2026 07:11 PM
MR No : 26071103492	Lab No : 47605	Ud. Lab. No : 46703
Name : Mahbubur Rahman	Age : 32 Y	Sex : M
Ref.By : n/		
Specimen : Serum	Counter : B1	
Investigation : Aspergillus Galactomannan Ag		
Method : Fluorescence Immunochromatography (Lateral flow assay)		

TEST RESULT

TEST	UNIT	RESULT	REFERENCE RANGE
Aspergillus Galactomannan Ag	Index	0.9	index < 0.5: Negative index ≥ 0.5: Positive

Interpretation :

- A) Negative results do not rule out the diagnosis of Invasive Aspergillosis. Repeat testing is recommended if the result is negative, but the disease is suspected.
- B) A positive result supports a diagnosis of invasive aspergillosis (IA). However, a single positive result should be interpreted with repeat testing on a new serum specimen. Positive results obtained with this assay should be considered in conjunction with clinical finding, microbiological culture, histological examination of biopsy samples and radiographic evidence.

Limitations :

- A) Resulted index value is not indicative of disease severity.
- B) The following conditions may cause false positive results:
1. Use of Beta-lactam antibiotics, such as piperacillin / Tazobactam, amoxicillin / clavulanic acid, cefepime etc.
 2. Cross reaction with other fungi: such as Histoplasma, Fusarium and Penicillium.
 3. Received blood products such as albumin, immunoglobulin, and coagulation factors.
 4. Neonatal Bifidobacterium Colonization.
 5. Cereals: Galactomannans are present in some cereals and enter the blood through damaged gut mucosa.
 6. Other factors: The presence of epitopes that cross-react with GM in cotton swab swabs used during surgery.
- C) The following conditions may cause false negative results
1. Preventive treatment against mold will significantly affect the GM test results.
 2. Local infections, including chronic granulomas.

Sutta

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INVESTIGATION REPORT

Lab/Manual No	: 2986051/F-3433	Order No	: OP27/353778
Patient Name	: MD. MAHBUBUR RAHMAN	UHID	: 1397563
Age/Gender	: 32 Y/M	Sample Collected	: 07/07/2026 11:55AM
Referred By	: AHSANIA MISSION CANCER & GENERAL HOSPITAL	Sample Received	: 07/07/2026 12:25PM
Ward/Bed No	: OPD	Report Date	: 07/07/2026 3:17PM
Clinical History	: bone marrow	Report Status	: Final

MOLECULAR DIAGNOSTICS LAB

IMMUNOPHENOTYPING-ACUTE LEUKEMIA

Type of sample: Bone marrow

The provided specimen presents with a predominant cell cluster in the blast region along with lymphocytes, monocytes and granulocytes.

Gating strategy : CD45 vs SSC
Events acquired : 50,000
Percentage cells gated : ~51% blast

On gating and further analysis, these critical cells express the following immunophenotyping findings –

Markers	Result (%)	Intensity	Interpretation
T-cell Markers			
cyCD3	—	Negative	Negative
sCD3	—	Negative	Negative
CD4	—	Negative	Negative
CD7	—	Negative	Negative
B-cell Markers			
CD19	—	Negative	Negative
CD79a	—	Negative	Negative
Myeloid Markers			
CD13	—	Negative	Negative
CD33	100%	Bright	Positive
CD117	—	Negative	Negative
CD14	—	Negative	Negative
CD64	100%	Moderate	Positive
CD15	100%	Moderate	Positive
MPO	90%	Dim to moderate	Positive

MD ABU HASAN
Chief Scientific Officer

Dr. MST RUMMANA RAHIM
MBBS, MPhil (Microbiology), M.Sc (Biotech)
ASSOCIATE CONSULTANT

Dr. MD MIZANUR RAHMAN
MBBS, PhD (Immunology)
SENIOR CONSULTANT - MOLECULAR DIAGNOSTICS
(BMDC No: A23462)

Prepared by: Dr. MD MIZANUR RAHMAN

Page 1 of 2



INVESTIGATION REPORT

Manual No	2986051/F-3433	Order No	10827/313773
Patient Name	MD. MAJBUBUR RAHMAN	UHID	1397563
Age / Gender	32 Y/M	Sample Collected	07/07/2026 11:55AM
Referred By	AHSANIA MISSION CANCER & GENERAL HOSPITAL	Sample Received	07/07/2026 12:25PM
Ward/Bed No	OPD	Report Date	07/07/2026 3:17PM
Clinical History	bone marrow	Report Status	Final

MOLECULAR DIAGNOSTICS LAB

IMMUNOPHENOTYPING-ACUTE LEUKEMIA

Markers	Result (%)	Intensity	Interpretation
Other Markers			
CD45	100%	Moderate	Positive
CD34	—	Negative	Negative
CD38	42%	Moderate	Positive
CD56	98%	Bright	Positive
HLA-DR	85%	Moderate	Positive

Result summary:

Blasts are positive for myeloid markers (CD15, CD33, CD64, MPO), CD56, CD38, immature marker CD34, HLA-DR and negative for CD13, CD14, CD34, CD117, B-cell and T-cell specific markers tested. Blast population aberrantly express CD56.

Impression:

The overall Immunophenotype findings of specimen provided are compatible with **Acute Myeloid Leukemia (AML)** with aberrant expression of **CD56**.

Please correlate with clinical features, peripheral blood findings, bone marrow morphology and cytogenetic/ molecular studies.

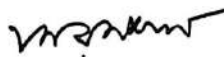
Advice: Leukemia genetic profile and Karyotyping for risk stratification.

Test platform:

Instrument : 12 color, 3 laser BD FACS Lyric (Becton Dickinson)
Method : Lysis followed by direct antibody staining.
— End of Report —


Dr. MD ABU HASAN
Chief Scientific Officer


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MBBS, MPhB (Microbiology) M.Sc (Biotech)
ASSOCIATE CONSULTANT


Dr. MD MIZANUR RAHMAN
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SENIOR CONSULTANT - MOLECULAR DIAGNOSTICS
(BMDC No: A22462)

Prepared by: Dr. MD MIZANUR RAHMAN

Page 2 of 2



IMM202607101555 HN. No.: RC2026070278 Collected on: 05-07-26 05:41:20 PM
7506504
05/07/2026 Received on: 05-07-26 06:41:20 PM
Patient's Name : MAHBUBUR RAHMAN Age : 20D/ 6 M/ 32 Y Delivery on : 05-07-26 08:14:04 PM
Sample : Blood Sex : Male
By : Dr. A. K. M. Jakaria, Consultant, ICU

IMMUNOLOGY TEST REPORT

(Relevant estimations are carried out by "VIDAS PC" Automated Analyzer & Checked manually.)

Test Name	Result	Unit/Opinion	Reference Range
High Sensitive Troponin-I (hs TnI)	584.5	ng/L	Normal: <19.0 ng/L Risk Zone: 19.0-99.0 ng/L AMI: >99.0 ng/L

Report Processor
Mr. Rubel Chakma


Md. Sarwar Jahan
DMLT(SMF)
Medical Technologist, AMCGH


Dr. Sk. Sharmin
MBBS(DU), M.Phil (Biochemistry)
Consultant, AMCGH

Report



No. : SER202607101554 HN # RC2026070278
No. : 7506504
Date : 05/07/2026
Patient's Name : MAHBUBUR RAHMAN
Age : 32 Y 6 M 20 D
Sex : Male
Ref. By : Dr. A. K. M. Jakaria, Consultant, ICU

SEROLOGICAL EXAMINATION REPORT

PROTHROMBIN TIME

Exam Name	Result	Unit	Normal Range
Patient	17.6 Sec.		10-17 Seconds
Control	12.1 Sec.		
Ratio	1.45:1		
Prothrombin Index	54.2 %		
INR	1.58		0.82-1.44 Warfarin Therapeutic Range: 2.0-3.5

Report Processor
Mr. Rubel Chakma


Md. Sarwar Jahan
DMLT(SMF)
Medical Technologist, AMCGH


Dr. Sk. Sharmin
MBBS(DU), M.Phil (Biochemistry)
Consultant, AMCGH



IMM202607101557 **HN. No.: RC2026070278** **Collected on: 05-07-26 05:41:20 PM**
7506504 **Received on: 05-07-26 06:41:20 PM**
05/07/2026 **Delivery on : 05-07-26 08:13:44 PM**
Name : MAHBUBUR RAHMAN **Age : 20D/ 6 M/ 32 Y**
Blood **Sex : Male**
Dr. A. K. M. Jakaria, Consultant, ICU

IMMUNOLOGY TEST REPORT

(Relevant estimations are carried out by "Geteln 1100" Automated Analyzer & Checked manually.)

Test Name	Result	Unit/Opinion	Reference Range
Procalcitonin (PCT)	>100.0	ng/ml	<0.1 ng/mL
NT-proBNP	18522	pg/ml	<300pg/ml, HF unlikely Age<50 Years, >450 pg/ml, HF likely Age50-75 Years, >900 pg/ml, HF likely Age>75Years, >1800 pg/ml, HF likely

Report Processor
Mr. Rubel Chakma

Sarwar
Md. Sarwar Jahan
DMLT(SMF)
Medical Technologist, AMCGH

Sharmin
Dr. Sk. Sharmin
MBBS(DU), M.Phil (Biochemistry)
Consultant, AMCGH



IMM202607101556

HN. No: RC2026070278

Collected on: 05-07-26 05:41:20 PM

7506504

Received on: 05-07-26 06:41:20 PM

05/07/2026

Delivery on : 05-07-26 08:13:32 PM

Name : MAHBUBUR RAHMAN

Age : 20D/ 6 M/ 32 Y

Blood

Sex : Male

: Dr. A. K. M. Jakaria, Consultant, ICU

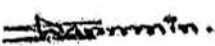
IMMUNOLOGY TEST REPORT

(Relevant estimations are carried out by "Geteln 1100" Automated Analyzer & Checked manually.)

Test Name	Result	Unit/Opinion	Reference Range
D-Dimer	>10.0	mg/l	< 0.5 mg/l

Report Processor
Mr Rubel Chakma


Md. Sarwar Jahan
DMLT(SMF)
Medical Technologist, AMCGH


Dr. Sk. Sharmin
MBBS(DU), M.Phil (Biochemistry)
Consultant, AMCGH

Report

: BIO7506504	HIN : RC2026070278	
: 7506504	Bill Date: 05/07/2026	Collected on: 05-07-26 05:41:20 PM
ss Name : MAHBUBUR RAHMAN	Age : 32 Y/ 6 M / 20 D	Received on: 05-07-26 06:41:20 PM
mon :	Sex : Male	Print on : 05-07-26 08:13:22 PM
By : Dr. A. K. M. Jakaria, Consultant, ICU		Type : Indoor , HDU-01(B-03)

BIOCHEMICAL TEST REPORT

(Relevant estimations were carried out by ""BECKMAN COULTER AU680"" Automated Analyzer & checked manually)

Test Name	Result	Unit	Reference Range
C-reactive protein	438.4	mg/L	< 10.0 mg/L

Report Processor
Mr. Rubel Chakma


Md. Sarwar Jahan
DMLT(SMF)
Medical Technologist ,AMCGH


Dr. Sk. Sharmin
MBBS(DU), M.Phil (Biochemistry)
Consultant ,AMCGH

BIO7506504

7506504

MAHBUBUR RAHMAN

Blood

Dr. A. K. M. Jakaria, Consultant, ICU

HIN : RC2026070278

Bill Date: 05/07/2020

Age : 32 Y / 6 M / 20 D

Sex : Male

Collected on: 05-07-20 05:41:20 PM

Received on: 05-07-20 06:41:20 PM

Print on : 05-07-20 08:13:22 PM

Type : Indoor , HDU-01(B-03)

BIOCHEMICAL TEST REPORT

(Relevant estimations were carried out by ""BECKMAN COULTER AU680"" Automated Analyzer & checked manually)

Test Name	Result	Unit	Reference Range
Urea	87	mg / dl	10-40 mg / dl
S. Magnesium	1.75	mg / dL	Children (2-12 years) : 1.7-2.3 mg / dL Adults (12-60 years) : 1.6-3.0 mg / dL
Creatinine	2.43	mg/dl	Adults- M: 0.6-1.20 mg/dl F: 0.5-1.10 mg/dl Neonates: 0.24-0.85 mg/dl 2-12 M: 0.17-0.42 mg/dl 1- <3y: 0.24-0.45 mg/dl 3- <5y: 0.31-0.47 mg/dl 5- <7y: 0.32-0.59 mg/dl 7-<9y: 0.40-0.60 mg/dl 9- <11y: 0.39- 0.73 mg/dl 11- <13y: 0.53-0.79 mg/dl 13- <15y: 0.57- 0.90 mg/dl
Calcium	6.00	mg/dl	Adults: 8.1-10.4 mg/dl Neonates(0-10days):7.60-10.20 mg/dl. children(2-12years): 8.1-10.8 mg/dl.
Albumin	2.10	g/ dl	3.2-5.0 g/ dl
LFT			
Bilirubin (Total)	0.99	mg/dl	Adults: up to 1.2 mg/dl Neonates: 24 hours: upto 8.0 mg/dl 48 hours: upto 13.0 mg/dl Children(>1 month): up to 1.0 mg/dl
SGPT	369	U/L	Male: up to 40 u/L Female: up to 31 u/L
Alkaline phosphatase	48	u/L	Male: 53-128 u/L female:42-98 u/L
SGOT	357	u/L	up to 40 u/L
Electrolytes/Na+K+Cl			
Sodium (Na+)	133	mmol/L	136-146 mmol/L
Potassium (K+)	3.10	mmol/L	3.5-5.1 mmol/L
Chloride (Cl)	98	mmol/L	101-109 mmol/L
HCO3	25	mmol/L	21-31 mmol/L

Report Processor
Mr. Rubel Chakma

Co. calcium 7.5
Sarwar
Md. Sarwar Jahan
DMLT(SMF)
Medical Technologist ,AMCGH

Sharmin
Dr. Sk. Sharmin
MBBS(DU), M.Phil (Biochemistry)
Consultant ,AMCGH

Ahsania Mission Cancer & General Hospital

(Machine Copy, Non Verified Report)

469
7506504/0240
SUBUR RAHMAN 32y
Comment:

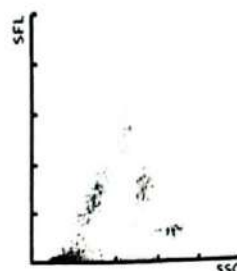
Adapter: 28
Ward: B-03

Pos.: 9 05/07/2026 18:23:36 WB
Doctor: Dr. A. K. M. Jakaria
Birth: 05/07/1994 Sex: Male
Nickname: XN-L

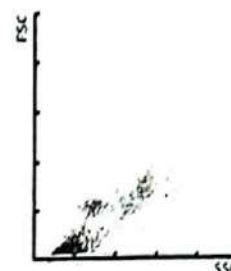
cive
Morph. Count

SC	1.03 *	[10 ³ /uL]	
RBC	4.15	[10 ⁶ /uL]	
HGB	11.3	[g/dL]	
HCT	32.7	[%]	
MCV	78.8 -	[fL]	
MCH	27.2	[pg]	
MCHC	34.6	[g/dL]	
PLT	77	[10 ³ /uL]	
RDW-SD	50.8	[fL]	
RDW-CV	17.5 +	[%]	
PDW	11.6	[fL]	
MPV	10.2	[fL]	
P-LCR	27.6	[%]	
PCT	0.08 -	[%]	
NEUT	0.47 *	[10 ³ /uL]	45.6 * [%]
LYMPH	0.35 *	[10 ³ /uL]	34.0 * [%]
MONO	0.13 *	[10 ³ /uL]	12.6 * [%]
EO	0.08 *	[10 ³ /uL]	7.8 * [%]
BASO	0.00 *	[10 ³ /uL]	0.0 * [%]
IG	0.00 *	[10 ³ /uL]	0.0 * [%]
MicroR	10.9	[%]	
MacroR	3.5	[%]	
WBC-BF		[10 ³ /uL]	
RBC-BF		[10 ⁶ /uL]	
MN		[10 ³ /uL]	[%]
PMN		[10 ³ /uL]	[%]
TC-BF#		[10 ³ /uL]	

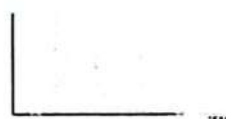
WDF



WDF-CBC



RBC



PLT



WBC IP Message
WBC Abn Scattergram
Neutropenia
Lymphopenia
Leukocytopenia
Blasts/Abn Lympho?
Left Shift?
Atypical Lympho?
NRBC?

RBC IP Message

PLT IP Message

Pt Absent F-45

Ahsania Mission Cancer & General Hospital, Uttara

A Non Profit Organization of Dhaka Ahsania Mission

Plot # 3, Embankment Driveway, Sector # 10, Uttara, Dhaka - 1230

Hotline: 10617, 09612310617, 01847359201, 0255092196-7, 0255092140

Email: info@amcghbd.org, Web: www.amcghbd.org, FB: www.facebook.com/ahsaniacancer

বহির্বিভাগীয় রোগীর চিকিৎসাপত্র

নং তারিখ : ৪.০৭.২৬
নাম : Mahbubur Rahman বয়স : ৩২/৩৩ লিঙ্গ : পুরুষ/মহিলা
ঠিকানা :

রোগ : DM E C K D E U T I

R_x

Ad

Admission in Hematology
ward/DC on ৭/৭/২৬
for Short/Long Chemo.,

Ad

Urine a/s,
CRP,
pro-calcitonin
plan:

BMS - NGS.

Flow

for Dr. Dr.
৭/৭/২৬.

Dr. A. Zubayer Khan
MBBS (DMC), FCPS (Hematology)
Specialist Hematologist
Assoc. Prof. & Head of Dept. of Hematology, NLMRC
Blood Disease, Blood Cancer & BMT Specialist
Ahsania Mission Cancer & General Hospital, Uttara

Oncology Patient's History

Registration No : 260704010325	Date : 04/07/2026	HN No RC2026070278	Gender : Male
Patient's Name : MAHBUBUR RAHMAN		Mobile No 01735741987	Age 32
Referral By		Marital Status : Married	Blood Gp. A+
Height(cm)	Weight(kg)	BSA (M2)	Pulse(/min)
			BP (mm of HG)
			Temp(F°)
			RR(/min)
Primary Diagnosis	: Bicytopenia- Under Evaluation		
Histopathology	: Chronic Large Granulocytic Leukemia(Probable Dx)-Bone Marrow Report [Possibility of Acute Myeloid Leukemia ?APML can not be ruled out -Bone marrow biopsy-19/06/2026] [PML-PARA translocation were not detected -Cytogenetics test-24/06/2026]. Date: 31-MAY-26		
Molecular Study Report	:		
Hormone Receptor status	:		
Immunohistochemistry/ Flowcytometry	:		
Cancer Code	:		
Metastases	:		
Associated with	: GB sludge+thick wall. Splenomegaly. Bell's Palsy, Haemolytic Jaundice.		
Stage of Disease	: Stage T N M		
Co morbidity	: DM,CKD		
Present Treatment Status	:		
Drug History	:		
Personal History	:		
Food Habits	:		
Family History	: Does anyone in the family have cancer ? : No		
Smoking History	: Non - Smoker		
Chewing Tobacco History	: Non - User		
Parity (in case of female)	:		
Contraceptive History	:		
Treatment Receive (Before reporting to AMCGH):	No		
Surgical History	:		
Chemotherapy History	:		
Radiotherapy History	:		
Tumour Markers	:		
Radiological Reports	: USG WA		
	24/05/26	Liver fatty(G-2). GB sludge+thick wall. Splenomegaly. Mod: Splenomegaly.	
Blood Reports	:		

Dr. A. M. Shahriar Sharif